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preliminarily enjoining its enforcement. *June Medical Services LLC v. Kliebert*, 158 F. Supp. 3d 473 (MD La.).

The State immediately asked the Court of Appeals for the Fifth Circuit to stay the District Court's injunction. The Court of Appeals granted that stay. But we then issued our own stay at the plaintiffs' request, thereby leaving the District Court's preliminary injunction (at least temporarily) in effect. See *June Medical Services, L. L. C. v. Gee*, 814 F. 3d 319 (CA5), vacated, 577 U. S. 1185 (2016).

Approximately two months later, in June 2016, we issued our decision in *Whole Woman's Health*, reversing the Fifth Circuit's judgment in that case. We remanded this case for reconsideration, and the Fifth Circuit in turn remanded the case to the District Court permitting it to engage in further factfinding. See *June Medical Services, L.L.C. v. Gee*, 2016 WL 11494731 (CA5, Aug. 24, 2016) (*per curiam*). All the parties agreed that the District Court could rule on the plaintiffs' request for a permanent injunction on the basis of the record it had already developed. Minute Entry in No. 14–cv–00525, Doc. No. 253. The court proceeded to do so.

D

Because the issues before us in this case primarily focus upon the factual findings (and fact-related determinations) of the District Court, we set forth only the essential findings here, giving greater detail in the analysis that follows.

With respect to the Act's asserted benefits, the District Court found that:

- “[A]bortion in Louisiana has been extremely safe, with particularly low rates of serious complications.” 250 F. Supp. 3d, at 65. The “testimony of clinic staff and physicians demonstrated” that it “rarely . . . is necessary to transfer patients to a hospital: far less than once a year, or less than one per several thousand patients.” *Id.*, at 63. And “[w]hether or not a patient’s treating

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physician has admitting privileges is not relevant to the patient's care." *Id.*, at 64.

- There was accordingly "'no significant health-related problem that the new law helped to cure.' . . . The record does not contain any evidence that complications from abortion were being treated improperly, nor any evidence that any negative outcomes could have been avoided if the abortion provider had admitting privileges at a local hospital." *Id.*, at 86–87. (quoting *Whole Woman's Health*, 579 U. S., at 610); see also 250 F. Supp. 3d, at 86–87 (summarizing conclusions).
- There was also "no credible evidence in the record that Act 620 would further the State's interest in women's health beyond that which is already insured under existing Louisiana law." *Id.*, at 65.

Turning to Act 620's impact on women's access to abortion, the District Court found that:

- Approximately 10,000 women obtain abortions in Louisiana each year. *Id.*, at 39. At the outset of this litigation, those women were served by six doctors at five abortion clinics. *Id.*, at 40, 41–44. By the time the court rendered its decision, two of those clinics had closed, and one of the doctors (Doe 4) had retired, leaving only Does 1, 2, 3, 5, and 6. *Ibid.*
- "[N]otwithstanding the good faith efforts of Does 1, 2, 4, 5 and 6 to comply with the Act by getting active admitting privileges at a hospital within 30 miles of where they perform abortions, they have had very limited success for reasons related to Act 620 and not related to their competence." *Id.*, at 78.
- These doctors' inability to secure privileges was "caused by Act 620 working in concert with existing laws and practices," including hospital bylaws and criteria that "preclude or, at least greatly discourage, the granting of privileges to abortion providers." *Id.*, at 50.

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- These requirements establish that admitting privileges serve no “‘relevant credentialing function’” because physicians may be denied privileges “for reasons unrelated to competency.” *Id.*, at 87 (quoting *Whole Woman’s Health*, 579 U. S., at 613).
- They also make it “unlikely that the [a]ffected clinics will be able to comply with the Act by recruiting new physicians who have or can obtain admitting privileges.” 250 F. Supp. 3d, at 82.
- Doe 3 testified credibly “that, as a result of his fears, and the demands of his private OB/GYN practice, if he is the last physician performing abortion in either the entire state or in the northern part of the state, he will not continue to perform abortions.” *Id.*, at 79; see also *id.*, at 78–79 (summarizing that testimony).
- Enforcing the admitting-privileges requirement would therefore “result in a drastic reduction in the number and geographic distribution of abortion providers, reducing the number of clinics to one, or at most two, and leaving only one, or at most two, physicians providing abortions in the entire state,” Does 3 and 5, who would only be allowed to practice in Shreveport and New Orleans. *Id.*, at 87. Depending on whether Doe 3 stopped practicing, or whether his retirement was treated as legally relevant, the impact would be a 55%–70% reduction in capacity. *Id.*, at 81.
- “The result of these burdens on women and providers, taken together and in context, is that many women seeking a safe, legal abortion in Louisiana will be unable to obtain one. Those who can will face substantial obstacles in exercising their constitutional right to choose abortion due to the dramatic reduction in abortion services.” *Id.*, at 88; see *id.*, at 79, 82, 87–88.
- In sum, “Act 620 does not advance Louisiana’s legitimate interest in protecting the health of women seeking abor-

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tions. Instead, Act 620 would increase the risk of harm to women’s health by dramatically reducing the availability of safe abortion in Louisiana.” *Id.*, at 87; see also *id.*, at 65–66.

The District Court added that

“there is no legally significant distinction between this case and [*Whole Woman’s Health*]: Act 620 was modeled after the Texas admitting privileges requirement, and it functions in the same manner, imposing significant obstacles to abortion access with no countervailing benefits.” *Id.*, at 88.

On the basis of these findings, the court held that Act 620 and its implementing regulations are unconstitutional. It entered an injunction permanently forbidding their enforcement.

E

The State appealed. A divided panel of the Court of Appeals reversed the District Court’s judgment. The panel majority concluded that Act 620’s impact was “dramatically less” than that of the Texas law invalidated in *Whole Woman’s Health*. *June Medical Services L. L. C. v. Gee*, 905 F. 3d 787, 791 (CA5 2018). “Despite its diligent effort to apply [*Whole Woman’s Health*] faithfully,” the majority thought that the District Court had “clearly erred in concluding otherwise.” *Id.*, at 815.

With respect to the Act’s asserted benefits, the majority thought that, “[u]nlike Texas, Louisiana presents some evidence of a minimal benefit.” *Id.*, at 805. Rejecting the District Court’s contrary finding, it concluded that the admitting-privileges requirement “performs a real, and previously unaddressed, credentialing function that promotes the wellbeing of women seeking abortion.” *Id.*, at 806. The majority believed that the process of obtaining privileges would help to “verify an applicant’s surgical ability,

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training, education, experience, practice record, and criminal history.” *Id.*, at 805, and n. 53. And it accepted the State’s argument that the law “brings the requirements regarding outpatient abortion clinics into conformity with the *preexisting* requirement that physicians at ambulatory surgical centers (‘ASCs’) must have privileges at a hospital within the community.” *Id.*, at 805.

Moving on to Act 620’s burdens, the appeals court wrote that “everything turns on whether the privileges requirement actually would prevent doctors from practicing in Louisiana.” *Id.*, at 807. Although the State challenged the District Court’s findings only with respect to Does 2 and 3, the Court of Appeals went further. It disagreed with nearly every one of the District Court’s findings, concluding that “the district court erred in finding that only Doe 5 would be able to obtain privileges and that the application process creates particular hardships and obstacles for abortion providers in Louisiana.” *Id.*, at 810. The court noted that “[a]t least three hospitals have proven willing to extend privileges.” *Ibid.* It thought that “only Doe 1 has put forth a good-faith effort to get admitting privileges,” while “Doe 2, Doe 5, and Doe 6 could likely obtain privileges,” *ibid.*, and “Doe 3’s personal choice to stop practicing cannot be legally attributed to Act 620,” *id.*, at 811.

Having rejected the District Court’s findings with respect to all but one of the physicians, the Court of Appeals concluded that “there is no evidence that Louisiana facilities will close from Act 620.” *Id.*, at 810. The appeals court allowed that the Baton Rouge clinic where Doe 5 had not obtained privileges would close. But it reasoned that “[b]ecause obtaining privileges is not overly burdensome, . . . the fact that one clinic would have to close is not a substantial burden that can currently be attributed to Act 620 as distinguished from Doe 5’s failure to put forth a good-faith effort.” *Ibid.* The Court of Appeals added that the additional work that Doe 2 and Doe 3 would have to do to compensate for Doe 1’s inabil-

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ity to perform abortions “does not begin to approach the capacity problem in” *Whole Woman’s Health*. 905 F. 3d, at 812. It estimated that Act 620 would “resul[t] in a potential increase” in waiting times “of 54 minutes at one of the state’s clinics for at most 30% of women.” *Id.*, at 815.

On the basis of these findings, the panel majority concluded that Louisiana’s admitting-privileges requirement would impose no “substantial burden at all” on Louisiana women seeking an abortion, “much less a substantial burden on a large fraction of women as is required to sustain a facial challenge.” *Ibid.* Judge Higginbotham dissented.

The Court of Appeals denied the plaintiffs’ petition for en banc rehearing over dissents by Judges Dennis and Higginson, joined by four of their colleagues. See *June Medical Services, L. L. C. v. Gee*, 913 F. 3d 573 (2019) (*per curiam*). The plaintiffs then asked this Court to stay the Fifth Circuit’s judgment. We granted their application, thereby allowing the District Court’s injunction to remain in effect. *June Medical Services, L. L. C. v. Gee*, 586 U. S. 1135 (2019). The plaintiffs subsequently filed a petition for certiorari addressing the merits of the appeals court’s decision. The State filed a cross-petition, challenging the plaintiffs’ authority to maintain this action. We granted both petitions.

II

We initially consider a procedural argument that the State raised for the first time in its cross-petition for certiorari. As we have explained, the plaintiff abortion providers and clinics in this case have challenged Act 620 on the ground that it infringes their patients’ rights to access an abortion. The State contends that the proper parties to assert these rights are the patients themselves. We think that the State has waived that argument.

The State’s argument rests on the rule that a party cannot ordinarily “rest his claim to relief on the legal rights or interests of third parties.” *Kowalski v. Tesmer*, 543 U. S.

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125, 129 (2004) (quoting *Warth v. Seldin*, 422 U. S. 490, 499 (1975)). This rule is “prudential.” 543 U. S., at 128–129. It does not involve the Constitution’s “case-or-controversy requirement.” *Id.*, at 129; see *Craig v. Boren*, 429 U. S. 190, 193 (1976); *Singleton v. Wulff*, 428 U. S. 106, 112 (1976). And so, we have explained, it can be forfeited or waived. See *Craig*, 429 U. S., at 193–194.

As we pointed out, *supra*, at 310, the State’s memorandum opposing the plaintiffs’ TRO request urged the District Court to proceed swiftly to the merits of the plaintiffs’ undue-burden claim. It argued that there was “no question that the physicians had standing to contest” Act 620. App. 44. And it told the District Court that the Fifth Circuit had found that doctors challenging Texas’ “identical” law “had third-party standing to assert their patients’ rights.” *Id.*, at 43–44. Noting that the Texas law had “already been upheld,” the State asserted that it had “a keen interest in removing any cloud upon the validity of its law.” *Id.*, at 45. It insisted that this suit was “the proper vehicle to do so.” *Ibid.* The State did not mention its current objection until it filed its cross-petition—more than five years after it argued that the plaintiffs’ standing was beyond question.

The State’s unmistakable concession of standing as part of its effort to obtain a quick decision from the District Court on the merits of the plaintiffs’ undue-burden claims bars our consideration of it here. See *Wood v. Milyard*, 566 U. S. 463, 474 (2012); cf. *post*, at 400–401 (ALITO, J., dissenting) (addressing the Court’s approach to claims forfeited rather than waived); *post*, at 415–416 (GORSUCH, J., dissenting) (addressing waiver of structural rather than prudential objections).

The State refers to the Fifth Circuit’s finding of standing in *Whole Woman’s Health* as an excuse for its concession. Brief for Respondent in No. 18–1323, p. 52 (Brief for Respondent). But the standing argument the State makes here rests on reasons that it tells us are specific to abortion providers in *Louisiana*. See *id.*, at 41–48. We are not per-

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suaed that the State could have thought it was precluded from making those arguments by a decision with respect to *Texas* doctors.

And even if the State had merely forfeited its objection by failing to raise it at any point over the last five years, we would not now undo all that has come before on that basis. What we said some 45 years ago in *Craig* applies equally today: “[A] decision by us to forgo consideration of the constitutional merits”—after “the parties have sought or at least have never resisted an authoritative constitutional determination” in the courts below—“in order to await the initiation of a new challenge to the statute by injured third parties would be impermissibly to foster repetitive and time-consuming litigation under the guise of caution and prudence.” 429 U. S., at 193–194 (quotation altered).

In any event, the rule the State invokes is hardly absolute. We have long permitted abortion providers to invoke the rights of their actual or potential patients in challenges to abortion-related regulations. See, e. g., *Whole Woman’s Health*, 579 U. S., at 592; *Gonzales*, 550 U. S., at 133; *Ayotte v. Planned Parenthood of Northern New Eng.*, 546 U. S. 320, 324 (2006); *Stenberg v. Carhart*, 530 U. S. 914, 922 (2000); *Mazurek v. Armstrong*, 520 U. S. 968, 969–970 (1997) (*per curiam*); *Casey*, 505 U. S., at 845 (majority opinion); *Akron v. Akron Center for Reproductive Health, Inc.*, 462 U. S. 416, 440, n. 30 (1983); *Planned Parenthood of Central Mo. v. Danforth*, 428 U. S. 52, 62 (1976); *Doe v. Bolton*, 410 U. S. 179, 188–189 (1973).

And we have generally permitted plaintiffs to assert third-party rights in cases where the “‘enforcement of the challenged restriction *against the litigant* would result indirectly in the violation of third parties’ rights.’” *Kowalski*, 543 U. S., at 130 (quoting *Warth*, 422 U. S., at 510); see, e. g., *Department of Labor v. Triplett*, 494 U. S. 715, 720 (1990) (Scalia, J., for the Court) (attorney raising rights of clients

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to challenge restrictions on fee arrangements); *Craig*, 429 U. S., at 192 (convenience store raising rights of young men to challenge sex-based restriction on beer sales); *Doe*, 410 U. S., at 188 (abortion provider raising the rights of pregnant women to access an abortion); *Carey v. Population Services Int'l*, 431 U. S. 678 (1977) (distributors of contraceptives raising rights of prospective purchasers to challenge restrictions on sales of contraceptives); *Eisenstadt v. Baird*, 405 U. S. 438 (1972) (similar); *Griswold v. Connecticut*, 381 U. S. 479, 481 (1965) (similar); *Sullivan v. Little Hunting Park, Inc.*, 396 U. S. 229 (1969) (white property owner raising rights of black contractual counterparty to challenge discriminatory restrictions on ability to contract); *Barrows v. Jackson*, 346 U. S. 249 (1953) (similar). In such cases, we have explained, “the obvious claimant” and “the least awkward challenger” is the party upon whom the challenged statute imposes “legal duties and disabilities.” *Craig*, 429 U. S., at 196–197; see *Akron*, 462 U. S., at 440, n. 30; *Danforth*, 428 U. S., at 62; *Doe*, 410 U. S., at 188.

The case before us lies at the intersection of these two lines of precedent. The plaintiffs are abortion providers challenging a law that regulates their conduct. The “threatened imposition of governmental sanctions” for non-compliance eliminates any risk that their claims are abstract or hypothetical. *Craig*, 429 U. S., at 195. That threat also assures us that the plaintiffs have every incentive to “resist efforts at restricting their operations by acting as advocates of the rights of third parties who seek access to their market or function.” *Ibid.* And, as the parties who must actually go through the process of applying for and maintaining admitting privileges, they are far better positioned than their patients to address the burdens of compliance. See *Singleton*, 428 U. S., at 117 (plurality opinion) (observing that “the physician is uniquely qualified to litigate the constitutionality of the State’s interference with, or discrimination against,”

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a woman's decision to have an abortion). They are, in other words, "the least awkward" and most "obvious" claimants here. *Craig*, 429 U. S., at 197.

Our dissenting colleagues suggest that this case is different because the plaintiffs have challenged a law ostensibly enacted to protect the women whose rights they are asserting. See *post*, at 401–402 (opinion of ALITO, J.); *post*, at 415 (opinion of GORSUCH, J.). But that is a common feature of cases in which we have found third-party standing. The restriction on sales of 3.2% beer to young men challenged by a drive-through convenience store in *Craig* was defended on "public health and safety" grounds, including the premise that young men were particularly susceptible to driving while intoxicated. 429 U. S., at 199–200; see Hager, Gender Discrimination and the Courts: New Ground to Cover, *Washington Post*, Sept. 26, 1976, p. 139. And the rule requiring approval from the Department of Labor for attorney fee arrangements challenged by a lawyer in *Triplett* was "designed to protect [their clients] from their improvident contracts, in the interest not only of themselves and their families but of the public." 494 U. S., at 722 (internal quotation marks omitted).

Nor is this the first abortion case to address provider standing to challenge regulations said to protect women. Both the hospitalization requirement in *Akron*, 462 U. S., at 435, and the hospital-accreditation requirement in *Doe*, 410 U. S., at 195, were defended as health and safety regulations. And the ban on saline amniocentesis in *Danforth* was based on the legislative finding "that the technique is deleterious to maternal health." 428 U. S., at 76 (internal quotation marks omitted).

In short, the State's strategic waiver and a long line of well-established precedents foreclose its belated challenge to the plaintiffs' standing. We consequently proceed to consider the merits of the plaintiffs' claims.

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III

A

Turning to the merits, we apply the constitutional standards set forth in our earlier abortion-related cases, and in particular in *Casey* and *Whole Woman's Health*. At the risk of repetition, we remind the reader of the standards we described above. In *Whole Woman's Health*, we quoted *Casey* in explaining that “‘a statute which, while furthering [a] valid state interest, has the effect of placing a substantial obstacle in the path of a woman’s choice cannot be considered a permissible means of serving its legitimate ends.’” 579 U. S., at 607 (quoting *Casey*, 505 U. S., at 877 (plurality opinion)). We added that “‘[u]nnecessary health regulations’” impose an unconstitutional “‘undue burden’” if they have “‘the purpose or effect of presenting a substantial obstacle to a woman seeking an abortion.’” 579 U. S., at 607 (quoting *Casey*, 505 U. S., at 878; emphasis added).

We went on to explain that, in applying these standards, courts must “consider the burdens a law imposes on abortion access together with the benefits those laws confer.” 579 U. S., at 607. We cautioned that courts “must review legislative ‘factfinding under a deferential standard.’” *Id.*, at 608 (quoting *Gonzales*, 550 U. S., at 165). But they “must not ‘place dispositive weight’ on those ‘findings,’” for the courts “‘retai[n] an independent constitutional duty to review factual findings where constitutional rights are at stake.’” 579 U. S., at 600 (quoting *Gonzales*, 550 U. S., at 165; emphasis deleted).

We held in *Whole Woman's Health* that the trial court faithfully applied these standards. It “considered the evidence in the record—including expert evidence, presented in stipulations, depositions, and testimony.” 579 U. S., at 609. It “then weighed the asserted benefits” of the law “against the burdens” it imposed on abortion access. *Ibid.* And it

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concluded that the balance tipped against the statute’s constitutionality. The District Court in this suit did the same.

B

The Court of Appeals disagreed with the District Court, not so much in respect to the legal standards that we have just set forth, but because it did not agree with the factual findings on which the District Court relied in assessing both the burdens that Act 620 imposes and the health-related benefits it might bring. Compare, *e. g., supra*, at 311–314, with *supra*, at 314–316. We have consequently reviewed the record in detail ourselves. In doing so, we have applied well-established legal standards.

We start from the premise that a district court’s findings of fact, “whether based on oral or other evidence, must not be set aside unless clearly erroneous, and the reviewing court must give due regard to the trial court’s opportunity to judge the witnesses’ credibility.” Fed. Rule Civ. Proc. 52(a)(6). In “‘applying [this] standard to the findings of a district court sitting without a jury, appellate courts must constantly have in mind that their function is not to decide factual issues *de novo*.’” *Anderson v. Bessemer City*, 470 U. S. 564, 573 (1985) (quoting *Zenith Radio Corp. v. Hazeltine Research, Inc.*, 395 U. S. 100, 123 (1969)). Where “the district court’s account of the evidence is plausible in light of the record viewed in its entirety, the court of appeals may not reverse it even though convinced that had it been sitting as the trier of fact, it would have weighed the evidence differently.” *Anderson*, 470 U. S., at 573–574. “A finding that is ‘plausible’ in light of the full record—even if another is equally or more so—must govern.” *Cooper v. Harris*, 581 U. S. 285, 293 (2017).

Our dissenting colleagues suggest that a different, less-deferential standard should apply here because the District Court enjoined the admitting-privileges requirement before it was enforced. See *post*, at 387–388 (opinion of ALITO, J.); *post*, at 419–422 (opinion of GORSUCH, J.). We are aware of no

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authority suggesting that appellate scrutiny of factual determinations varies with the timing of a plaintiff's lawsuit or a trial court's decision. And, in any event, the record belies the dissents' claims that the District Court's findings in this case were "conjectural" or premature. As we have explained, the District Court's order on the plaintiffs' motion for a temporary restraining order suspended only Act 620's penalties. The plaintiffs were required to continue in their efforts to obtain admitting privileges. See *supra*, at 310. The District Court supervised those efforts through the trial and beyond. See 250 F. Supp. 3d, at 77. It based its findings on this real-world evidence, not speculative guesswork. Nor can we agree with the suggestion that the timing of the District Court's decision somehow prejudiced the State. From the start, the State urged that the District Court decide the merits of the plaintiffs' claims without awaiting a decision on their applications for admitting privileges. See App. 43–44. And, when this case returned to the District Court in August 2016, following our decision in *Whole Woman's Health*, the State stipulated that the case was ripe for decision on the record as it stood in June 2015. See *supra*, at 310–311. In short, we see no legal or practical basis to depart from the familiar standard that applies to all "[f]indings of fact." Fed. Rule Civ. Proc. 52(a).

Under that familiar standard, we find that the testimony and other evidence contained in the extensive record developed over the 6-day trial support the District Court's ultimate conclusion that, "[e]ven if Act 620 could be said to further women's health to some marginal degree, the burdens it imposes far outweigh any such benefit, and thus the Act imposes an unconstitutional undue burden." 250 F. Supp. 3d, at 88.

IV

The District Court's Substantial-Obstacle Determination

The District Court found that enforcing the admitting-privileges requirement would "result in a drastic reduction

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in the number and geographic distribution of abortion providers.” *Id.*, at 87. In light of demographic, economic, and other evidence, the court concluded that this reduction would make it impossible for “many women seeking a safe, legal abortion in Louisiana . . . to obtain one” and that it would impose “substantial obstacles” on those who could. *Id.*, at 88. We consider each of these findings in turn.

A

Act 620’s Effect on Abortion Providers

We begin with the District Court’s findings in respect to Act 620’s impact on abortion providers. As we have said, the court found that the Act would prevent Does 1, 2, and 6 from providing abortions. And it found that the Act would bar Doe 5 from working in his Baton Rouge-based clinic, relegating him to New Orleans. See *supra*, at 312–313.

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In *Whole Woman’s Health*, we said that, by presenting “direct testimony” from doctors who had been unable to secure privileges, and “plausible inferences to be drawn from the timing of the clinic closures” around the law’s effective date, the plaintiffs had “satisfied their burden” to establish that the Texas admitting-privileges requirement caused the closure of those clinics. 579 U. S., at 614.

We wrote that these inferences were bolstered by the submissions of *amici* in the medical profession, which “describe[d] the undisputed general fact that hospitals often” will restrict admitting privileges to doctors likely to seek a “certain number of admissions per year.” *Id.*, at 612 (internal quotation marks omitted). The likely effect of such requirements was that abortion providers “would be unable to maintain admitting privileges or obtain those privileges for the future, because the fact that abortions are so safe meant that providers were unlikely to have any patients to admit.” *Id.*, at 613. We also referred to “common prerequisites to

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obtaining admitting privileges that have nothing to do with ability to perform medical procedures”; for example, requirements that doctors have “treated a high number of patients in the hospital setting in the past year, clinical data requirements, residency requirements, and other discretionary factors.” *Ibid.*

To illustrate how these criteria impacted abortion providers, we noted the example of an obstetrician with 38 years’ experience who had been denied admitting privileges for reasons “‘not based on clinical competence considerations.’” *Ibid.* This, we said, showed that the law served no “relevant credentialing function,” but prevented qualified providers from serving women who seek an abortion. *Ibid.* And that, in turn, “help[ed] to explain why the new [law’s admitting-privileges] requirement led to the closure of” so many Texas clinics. *Id.*, at 612.

The evidence on which the District Court relied in this case is even stronger and more detailed. The District Court supervised Does 1, 2, 5, and 6 for over a year and a half as they tried, and largely failed, to obtain conforming privileges from 13 relevant hospitals. See 250 F. Supp. 3d, at 77–78; App. 48–55, 64–82. The court heard direct evidence that some of the doctors’ applications were denied for reasons that had nothing to do with their ability to perform abortions safely. 250 F. Supp. 3d, at 68–70, 76–77; App. 1310, 1435–1436. It also compiled circumstantial evidence that explains why other applications were denied and explains why, given the costs of applying and the reputational risks that accompany rejection, some providers could have chosen in good faith *not* to apply to every qualifying hospital. *Id.*, at 1135, 1311 (discussing the costs associated with unsuccessful applications). That circumstantial evidence includes documents and testimony that described the processes Louisiana hospitals follow when considering applications for admitting privileges, including requirements like the ones we cited in *Whole Woman’s Health* that are unrelated to a doctor’s competency

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to perform abortions. See generally Brief for Medical Staff Professionals as *Amici Curiae* 11–30 (reviewing the hospital bylaws in the record).

The evidence shows, among other things, that the fact that hospital admissions for abortion are vanishingly rare means that, unless they also maintain active OB/GYN practices, abortion providers in Louisiana are unlikely to have any recent in-hospital experience. 250 F. Supp. 3d, at 49. Yet such experience can well be a precondition to obtaining privileges. Doe 2, a board-certified OB/GYN with nearly 40 years' experience, testified that he had not "done any in-hospital work in ten years" and that just two of his patients in the preceding 5 years had required hospitalization. App. 387, 400. As a result, he was unable to comply with one hospital's demand that he produce data on "patient admissions and management, consultations, and procedures performed" in-hospital before his application could be "processed." *Id.*, at 1435; see *id.*, at 437–438. Doe 1, a board-certified family doctor with over 10 years' experience, was similarly unable to "submit documentation of hospital admissions and management of patients." *Id.*, at 1436.

The evidence also shows that many providers, even if they could initially obtain admitting privileges, would be unable to keep them. That is because, unless they have a practice that requires regular in-hospital care, they will lose the privileges for failing to use them. Doe 6, a board-certified OB/GYN practitioner with roughly 50 years' experience, provides only medication abortions. *Id.*, at 1308. Of the thousands of women he served over the decade before the District Court's decision, during which he also performed surgical abortions, just two required a direct transfer to a hospital and one of them was treated without being admitted. *Id.*, at 1309. That safety record would make it impossible for Doe 6 to maintain privileges at any of the many Louisiana hospitals that require newly appointed physicians to undergo a process of "focused professional practice evalua-

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tion,” in which they are observed by hospital staff as they perform in-hospital procedures. See Record 2635, 2637, 2681, 9054; Brief for Medical Staff Professionals as *Amici Curiae* 28–29 (describing this practice); cf. Record 10755 (requiring an “on-going review” of practice “in the Operating Room”). And it would likewise disqualify him at hospitals that require physicians to admit a minimum number of patients, either initially or on an ongoing basis. See, e. g., *id.*, at 9040, 9068–9069, 9150–9153; cf. App. 1193, 1182 (provider with no patient contacts in first year assigned to “Affiliate” status, without admitting privileges).

The evidence also shows that opposition to abortion played a significant role in some hospitals’ decisions to deny admitting privileges. 250 F. Supp. 3d, at 48–49, 51–53 (collecting evidence). Some hospitals expressly bar anyone with privileges from performing abortions. App. 1180, 1205. Others are unwilling to extend privileges to abortion providers as a matter of discretion. *Id.*, at 1127–1129. For example, Doe 2 testified that he was told not to bother asking for admitting privileges at University Health in Shreveport because of his abortion work. *Id.*, at 383–384. And Doe 1 was told that his abortion work was an impediment to his application. *Id.*, at 1315–1316.

Still other hospitals have requirements that abortion providers cannot satisfy because of the hostility they face in Louisiana. Many Louisiana hospitals require applicants to identify a doctor (called a “covering physician”) willing to serve as a backup should the applicant admit a patient and then for some reason become unavailable. See Record 9154, 9374, 9383, 9478, 9667, 10302, 10481, 10637, 10659–10661, 10676. The District Court found “that opposition to abortion can present a major, if not insurmountable hurdle, for an applicant getting the required covering physician.” 250 F. Supp. 3d, at 49; cf. *Whole Woman’s Health*, 579 U. S., at 613 (citing testimony describing similar problems faced by Texas providers seeking covering physicians). Doe 5 is a

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board-certified OB/GYN who had been practicing for more than nine years at the time of trial. Of the thousands of abortions he performed in the three years prior to the District Court's decision, not one required a direct transfer to a hospital. App. 1134. Yet he was unable to secure privileges at three Baton Rouge hospitals because he could not find a covering physician willing to be publicly associated with an abortion provider. *Id.*, at 1335–1336. Doe 3, a board-certified OB/GYN with nearly 45 years of experience, testified that he, too, had difficulty arranging coverage because of his abortion work. *Id.*, at 200–202.

Just as in *Whole Woman's Health*, the experiences of the individual doctors in this case support the District Court's factual finding that Louisiana's admitting-privileges requirement, like that in Texas' law, serves no "relevant credentialing function." 250 F. Supp. 3d, at 87 (quoting *Whole Woman's Health*, 579 U. S., at 613).

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The Court of Appeals found another explanation for the doctors' inability to obtain privileges more compelling. It conceded that Doe 1 would not be able to obtain admitting privileges in spite of his good-faith attempts. It concluded, however, that Does 2, 5, and 6 had acted in bad faith. 905 F. 3d, at 807. The problem is that the law requires appellate courts to review a trial court's findings under the deferential clear-error standard we have described. See *supra*, at 322–323. Our review of the record convinces us that the Court of Appeals misapplied that standard.

JUSTICE ALITO does not dispute that the District Court's findings are not "clearly erroneous." He argues instead that both the District Court and the Court of Appeals applied the wrong legal standard to the record in this case. By asking whether the doctors acted in "good faith," he contends, the courts below failed to account for the doctors' supposed "incentive to do as little as" possible to obtain conforming privi-

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leges. *Post*, at 388–390 (dissenting opinion); cf. *post*, at 419–420 (GORSUCH, J., dissenting). But that is not a legal argument at all. It is simply another way of saying that the doctors acted in *bad* faith. The District Court, after monitoring the doctors’ efforts for a year and a half, found otherwise. And “[w]hen the record is examined in light of the appropriately deferential standard, it is apparent that it contains nothing that mandates a finding that the District Court’s conclusion was clearly erroneous.” *Anderson*, 470 U. S., at 577.

Doe 2

The District Court found that Doe 2 tried in good faith to get admitting privileges within 30 miles of his Shreveport-area clinic. 250 F. Supp. 3d, at 68. The Court of Appeals thought that conclusion clearly erroneous for three reasons.

First, the appeals court suggested that Doe 2 failed to submit the data needed to process his application to Bossier’s Willis-Knighton Health Center. 905 F. 3d, at 808. It is true that Doe 2 submitted no additional information in response to the last letter he received from Willis-Knighton. But the record explains that failure. Doe 2 reasonably believed there was no point in doing so. The hospital’s letter explained that the data Doe 2 had already “submitted supports the outpatient [abortion] procedures you perform[ed].” App. 1435. But, the letter added, this data did “not support your request for hospital privileges” because it did not allow the hospital to “evaluate patient admissions and management, consultations, and procedures performed.” *Ibid.* Doe 2 testified at trial that he understood this to mean that he would have to submit records of *hospital* admissions, even though he had not “done any in-hospital work in ten years.” *Id.*, at 387; see *id.*, at 437 (“I’ve explained that that information doesn’t exist”). Doe 2’s understanding was consistent with Willis-Knighton’s similar letter to Doe 1, which explicitly stated that “we require that you submit documentation of hospital admissions and management of patients”

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Id., at 1436. The record also shows that Doe 2 could not have maintained the “adequate number of inpatient contacts” Willis-Knighton requires to support continued privileges. Record 9640; see App. 387–390, 404. JUSTICE ALITO faults Doe 2 for failing to pursue an application for “courtesy staff” privileges. See *post*, at 394. For one thing, it is far from clear that courtesy privileges entitle a physician to admit patients, as Act 620 requires. Compare, *e. g.*, Record 9640 with *id.*, at 9643. For another, that would not solve the problem that Doe 2 lacked the required in-hospital experience. JUSTICE ALITO wonders whether Willis-Knighton might have conferred courtesy privileges even without that experience. But the factors the hospital considers for both tiers of privileges are facially identical. *Id.*, at 9669. We have no license to reverse a trial court’s factual findings based on speculative inferences from facts not in evidence.

Second, the Court of Appeals found Doe 2’s explanation that Christus Schumpert Hospital “would not staff an abortion provider” to be “blatantly contradicted by the record.” 905 F. 3d, at 808. The record, however, contains Christus’ bylaws. They state that “[n]o activity prohibited by” the Ethical and Religious Directives to which the hospital subscribes “shall be engaged in by any Medical Staff appointee or other person exercising clinical privileges at the Health System.” App. 1180. These directives provide that abortion “is never permitted.” *Id.*, at 1205. And they warn against “the danger of scandal in any association with abortion providers.” *Ibid.*

The State suggests that the Court of Appeals, in speaking of a “contradic[tion],” was referring to the fact that Doe 3 had admitting privileges at Christus, as had Doe 2 at an earlier time. Brief for Respondent 75. Doe 3 testified, however, that he did not know whether Christus was “aware that I was performing abortions” and that he did not “feel like testing the waters there”—*i. e.*, by “asking [Christus] how they would feel” if they were aware that he “was performing